

OAKWOOD MEDICAL CENTER

Geriatric Medicine & Memory Care Unit

DISCHARGE SUMMARY

Document No: DS-2024-02190	Admission Date: March 13, 2024	Discharge Date: March 18, 2024
Attending Physician: Dr. Patricia Huang, MD, CMD		

Patient Name: Eleanor Jean Fontaine	Date of Birth: June 11, 1938 (Age: 85)
MRN: OMC-558-1147	Sex: Female
Insurance: Medicare + Medicaid (dual eligible)	Discharge Disposition: Memory Care Unit — Brightwood Gardens

COGNITIVE STATUS NOTE

Patient has moderate-to-severe Alzheimer's dementia (FAST Stage 6).

Patient lacks capacity for medical decision-making.

Healthcare Proxy: Son — Thomas Fontaine (240-555-7731). Must be contacted for all significant care decisions.

Patient may be agitated with unfamiliar caregivers — use calm, slow, reassuring communication.

Do NOT use physical restraints — behavioral approach required per psychiatry consult.

[!] MOLST (Medical Orders for Life-Sustaining Treatment) form completed — DNR/DNI status. Copy must travel with patient and be prominently placed in chart.

PRIMARY DIAGNOSIS

Urinary tract infection (UTI) — E. coli, uncomplicated

Secondary: Moderate-to-severe Alzheimer's dementia, Atrial fibrillation (chronic), Osteoporosis, Urinary incontinence, Malnutrition (mild)

REASON FOR ADMISSION

Patient brought in by memory care staff on 3/13 with acute behavioral change — increased confusion, agitation, and refusal to eat/drink over 2 days. Urinalysis positive for UTI. Urine culture grew E. coli — pan-sensitive. No fever. Patient treated with IV antibiotics x2 days then transitioned to oral. Behavioral symptoms resolved with treatment of underlying infection.

HOSPITAL COURSE

IV ceftriaxone initiated on admission. Transitioned to oral trimethoprim/sulfamethoxazole (TMP-SMX) on day 3 per sensitivity results. Nutrition consult obtained — patient has 8% weight loss over 3 months, recommend high-calorie supplements. Occupational therapy assessed for feeding — patient requires hand-over-hand assistance. Geriatric psychiatry consulted for agitation management — non-pharmacologic approach recommended first-line.

DISCHARGE MEDICATIONS

Medication	Dose	Route	Frequency	Notes
TMP-SMX DS (NEW)	1 tablet	PO	Twice daily x5 more days	Complete antibiotic course — ends 3/23
Donepezil	10mg	PO	Once daily at bedtime	Crush and mix with food if needed
Memantine	10mg	PO	Twice daily	Crush and mix with food if needed

Apixaban	2.5mg	PO	Twice daily	For AFib — reduced dose (age/weight criteria)
Metoprolol Succinate	25mg	PO	Once daily	Rate control — AFib
Calcium + Vitamin D3	600mg/400IU	PO	Twice daily	With meals
Alendronate (ON HOLD)	—	—	HELD	Held — patient cannot sit upright 30min post-dose
Melatonin (NEW)	3mg	PO	At bedtime	Non-pharmacologic sleep support
Mirtazapine (NEW)	7.5mg	PO	At bedtime	Appetite stimulant + sleep — START TONIGHT

BEHAVIORAL & CARE NOTES

Patient responds well to music (1940s-50s era — family confirmed). Use during meals and care routines.

Preferred morning routine: wash face first, then breakfast, then dressing.

Patient becomes distressed with rushed or forceful care — allow extra time for all ADLs.

Finger foods preferred — patient has difficulty with utensils.

Family visits regularly — son Thomas visits every Sunday. Include him in care plan meetings.

[!] Apixaban 2.5mg twice daily: confirm facility can administer this dose — some pharmacies only stock 5mg tablets.

[!] TMP-SMX and apixaban interaction: increased bleeding risk — monitor for bruising and bleeding.

[!] Alendronate on hold: no plan documented for resumption — confirm with PCP.

NUTRITION ORDERS

High-calorie, high-protein diet — minimum 1800 kcal/day.

Oral nutritional supplement: Ensure Plus or equivalent — twice daily with meals.

Texture: Soft/minced — no hard or chewy foods.

Weigh weekly — notify provider if weight loss >2 lbs/week.

Recheck albumin and prealbumin in 4 weeks.

[!] Nutritional supplement (Ensure Plus) order NOT placed — facility to arrange.

FOLLOW-UP

1. PCP (Dr. Huang): 2 weeks — 240-555-9900
2. Urine culture repeat: 1 week post-antibiotic completion (3/30)
3. Nutritionist: 4 weeks — facility to arrange
4. Family care conference: within 1 week of admission — include son Thomas

[!] MOLST document must be reviewed with family within 72 hours of facility admission.

ALLERGIES

Haldol (haloperidol) — severe extrapyramidal symptoms

Penicillin — rash